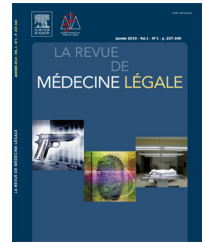




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ORIGINAL ARTICLE

The profile of women victims of domestic violence in Morocco

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Summary

Introduction. — Violence against women by an intimate partner is a real public health problem. Its prevalence is alarming.

Purpose. — To determine the profile of both the women victims of violence and their partners, and to describe the characteristics of violence against them.

Materials and methods. — Two hundred and fifty-four women were received during the study's period in our forensic unit for a forensic certificate. A pre-questionnaire was led by the authors. Participation in the study was voluntary and anonymous. The results were statistically analyzed. **Results.** — Most of the victims were young and had an average educational and economic level. They were beaten by a partner who takes toxic in more than three quarter of the cases. Physical violence was one of act reported by most women.

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Introduction

Violence against women is considered a main public health problem in most countries of the world. It can have serious consequences on women's health [1,2]. Among violence forms practiced against women, domestic violence or spousal violence that was highlighted in all countries and in all socio-economic backgrounds, supporting documents, and available evidence shows that it is much more widespread than imagined. In different regions of the world, between 16 and 52% of women are victims of physical violence by their partner [3].

This conjugal violence (CV) is a complex set of behaviors. It may include, in addition to physical violence, sexual, psychological, emotional or verbal [4].

The awareness of the problem's extent is complex, even in communities that are highly aware of the phenomenon, most published studies on the CV were held in medical community. Around the world, the number of women victims of CV is alarming. In Europe, the prevalence is between 20 and 55% [1]. Studies in the United States of America have shown that the prevalence of physical violence against women by an intimate partner is between 28 and 54.2% annually [4]. In a multicenter study held in several countries in Africa, Asia, Europe and South America, the prevalence of intraconjugal violence was 15 to 71% [5].

On the other hand, several studies support that there are a combination of other factors with this violence and that

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their understanding is much more important because it will help to better understand the phenomenon of violence against women and so to treat and prevent it [2]. Several factors have been described in the literature as risk factors for this phenomenon namely: the young age of the woman and her partner, the low socio-economic and intellectual level, drug and ethyl liquor use by woman or her partner, psychiatric disease with personality problems, criminal history, cultural dynamics and many other factors. . . Besides, it should be noted that any woman regardless of her educational and socio-economic level can suffer from domestic violence [2].

In Morocco, the official information on the number of beaten women are mainly provided by the police, justice and non-governmental organizations across the cases treated and the toll free number available to women victims of violence based on genre. A recent report by the High Planning Commission in 2011 showed that the prevalence of domestic violence was 55% [6].

Aware of the problem of violence against women, Morocco has set up a national strategy to struggle with violence against women in 2002 with an operational schedule (2005) which has set as the main objective the creation of medico-psychological and forensic unit care of women victims of violence in hospitals. Women victims of violence management is part of an integrated network management involving four partners: health sector, judicial system (courts, police and royal police force), civil society by associations and listening centers for psychological support and legal assistance to women victims of violence and child protection units.

In the same time, the Moroccan legislator set on several arrangements concerning the struggle against all discrimination types, including that based on gender. Indeed, if national legislation does not have a clear definition of discrimination, the fact remains that many prescriptive arrangements recognize the equal rights of men and women to have their human rights and basic freedoms, and that, regardless of their social status. Equality establishment is becoming the rule. Testify towards this, the hole redesign of the Family Code, the new legislation governing civil status, the penal code arrangements and work against discrimination, whatever its origins.

Hence to protect more women victims of violence, the legislator gave permission - without obligation - to the doctor to report to the competent judicial or administrative authorities, the criminal and abuse acts or deprivation perpetrated against a woman; by exempting from proceeding for violation of professional secret (Article 446 of the Criminal Code in its second paragraph) [7].

This work's main purpose is to determine the profile of both women victims of violence and their partners, and to describe the characteristics of violence against women.

Materials and methods

This is a prospective study held along a period of six months in the forensic department of the University Hospital Ibn Rochd Casablanca on women victims of domestic violence who seek the management unit for women and children victims of violence.

The study variables were collected on a survey sheet made for this purpose. It included questions on sociodemographic characteristics of the woman and her partner (age, education, occupation); the couple's income (<or> 3000 Dh the guaranteed minimum wage (GMW) "260 euro"); marital status (married, divorced, fiancée); the children's number, toxic habits, psychiatric and criminal history; as well as the variables related to the characteristics of marriage (ground, length) and lived violent episode. Domestic violence has been classified as physical, sexual or psychological. Physical violence was defined as any positive response any time the woman reported the following actions: "be dragged or beaten by blows or object/instrument, pushed, choked, burned, shaken...". About sexual violence, it was defined as any positive response to the following questions "being forced to have degrading or humiliating sex (sodomy for example)", "it is subject to sex for fear" or "he forces her to have sex".

Finally, psychological violence was defined as a positive response if the woman was already being threatened, intimidated, humiliated or belittled, insulted by her partner.

After anonymisation, data were collected in a Microsoft Excel[®] table. Statistical analysis was performed using the SPSS[®] 13.0 software. Quantitative variables were expressed as average $\pm$ standard deviation or median and quartiles and categorical variables were expressed as numbers and percentages. Comparisons were made by Student's t test or Mann Whitney and in the case of quantitative variables. The chi 2 test or exact test in the case of qualitative variables. The significance level was set at 5%.

Each woman who participated in our work was informed about the study and its objectives and has written consent.

Results

During the study period, 254 women were identified. The average age was 34 ± 9 years. Regarding the studies level, 140 patients (57.4%) had the secondary level and 48 (19.7%) were illiterate. Ninety-nine patients (40%) had a professional activity and 8 (3%) had toxic habits. One hundred and ten (63%) couples had incomes above the minimum wage. It was noted that 187 (75.1%) of the couples had at least 2 children (Table 1). As for the aggressor, he was the husband in 249 (98%) of the cases, the ex-spouse in 3 cases (1.2%) and the engaged in 2 (0.8%) with a median age of 40 and interquartile range [33–47]. Twenty-seven (18.5%) of the aggressors were illiterate. He had a job in 199 (80.6%) of the cases, toxic habits in 196 (78%) of the cases, psychiatric history in 15 (6%) of the cases and criminal history in 60 (24.7%) of the cases. At the time of the attack reported by the victim, 196 (82%) of the attackers were taking drugs or alcohol (Table 1).

For married couples, their marriage was by love in 78 (53%) of the cases, by family arrangement in 61 of the cases (41.5%) and in 8 of the cases out of necessity or 5.4%. The median number of years of marriage was 7 years with an interquartile range [4–14].

All women have suffered from physical violence that was unique in 129 (51%) of the cases. In 190 of the cases (88%), violence has occurred in the family home from one to several times a month in 191 (84.5%) of the cases. Abused women

Table 1 Sociodemographic characteristics of women victims of domestic violence and their abusers.

Characteristic	Women (N) (percentage %)
Age ^a	
< 25 years	41 (16.5)
25–35	106 (42.6)
> 35	102 (41)
Matrimonial status ^a	
Married	245 (96.8)
Divorced	4 (1.6)
Engaged	4 (1.6)
Educational level ^a	
Illiterate	48 (19.7)
Primary	56 (23)
Secondary	140 (57.4)
Job ^a	
Yes	99 (40)
No	148 (60)
Toxic habits ^a	
Yes	8 (3.1)
No	246 (96.9)
Number of children ^a	
≤ 2	187 (75.1)
> 2	62 (24.9)
Couple's wage ^a	
≤ GMW	187 (75.1)
> GMW	62 (24.9)
Age ^b	
< 25 years	6 (2.5)
25–35	71 (30.1)
> 35	236 (67.4)
Educational level ^b	
Illiterate	27 (18.5)
Primary	34 (29.5)
Secondary	69 (47.3)
Superior	7 (4.8)
Job ^b	
Yes	199 (80.6)
No	48 (19.4)
Toxic habits ^b	
Yes	196 (78)
No	55 (22)
Psychiatric history ^b	
Yes	15 (6.1)
No	229 (93.1)
Criminal history ^b	
Yes	60 (24.7)
No	182 (74.9)
The aggressor state at the time of attack ^b	
Normal	43 (18)
Use of toxic or alcohol	196 (82)

^a Of woman.^b Of the abuser.

the husband in 72 (32%) of the cases and money problems in 41 (18%) of the cases. Concerning children, they witnessed the violence on their mother in 195 of the cases (93%) and they were beaten together with their mothers in 115 of the cases (66.5%) (Table 2).

In 68 cases (31.3%), the victim had no reaction against the violence suffered. She had a forensic certificate without judicial follow up in 39 of the cases (16.7%), filed a complaint in 8 cases and 3.4% divorced in 9 cases (3.9%).

Contusions of the 1st and 2nd degree (bruising/hematoma) lesions were the most scored in 155 of the cases (91.7%). Twenty-five cases (10.6%) of dislocations and fractures have been reported, 20 cases or 9.1% of burns and 12 cases (5.1%) of amputations. The average duration of work disability was 19 days ± 6. The association analysis of the typology of violence and sociodemographic factors showed that low income (< minimum wage) was significantly associated to the occurrence of physical violence only against physical violence associated to other types $P = 0.03$ (Table 3).

Daily aggression was associated as statistics are concerned significantly with age ($P = 0.01$), the victim's educational level ($P = 0.04$) and the couple's low income ($P = 0.04$). Indeed, it was noticed that the attack was repeated daily in younger women under 25 years, among the illiterate and those with couples earning less than or equal to the minimum wage (Table 4).

Table 2 Characteristics of the violence suffered.

Characteristic	Women (N) (percentage %)
Place	
Conjugal home	190 (88)
Other homes	20 (9.3)
Public way	5 (2.3)
Public place	1 (0.5)
Violence type	
Physical alone	129 (51)
Physical and psychological	90 (45.5)
Physical and sexual	13 (5.1)
Physical, psychological and sexual	13 (5.1)
Violence's beginning	
During the 1st year	177 (73.4)
Beyond the 1st year	64 (26.6)
Beginning meeting with	
Pregnancy	69 (39.2)
Adultery	21 (11.9)
No work	9 (5.1)
Don't know	107 (43.7)
Violence frequency	
Daily	35 (15.5)
One to more times per month	191 (84.5)
Reason	
Pecuniary	41 (18.1)
Law's problem	18 (7.9)
Sexual problem	17 (7.5)
The husband's aggressiveness	72 (31.7)
Problem related to children	22 (9.6)
Many reasons	87 (25.2)

reported that the start of the violence occurred during the first year in 177 of the cases (73.4%) and it coincided with pregnancy in 69 (39%) of the cases. The main reasons for this violence, according to women, were the aggressiveness of

Table 3 Comparison of physical violence alone versus multiple violence depending on woman and her aggressor's characteristics.

Characteristic	Physical violence alone (Women (N) noticed %)	Associated violence (Women (N) noticed %)	P
Age ^a			0.6
< 25 years	43.9	56.1	
25-35	49.1	50.9	
> 35	52	48	
Matrimonial status ^a			0.6
Married	49.6	50.4	
Divorced	50	50	
Engaged	25	75	
Educational level ^a			0.12
Illiterate	60.4	39.6	
Primary	41.1	58.9	
Secondary	46.8	53.2	0.6
Job ^a			
Yes	51.5	48.5	
No	48.3	51.7	
Couple's wage			0.03
≤ GMW	30.7	34.3	
> GMW	52.3	58.7	
Children's number			0.5
≤ 2	51.1	48.9	
> 2			
Age ^b			0.9
< 25 years	50	50	
25-35	48.6	51.4	
> 35	49.7	50.3	
Educational level ^b			0.6
Illiterate	55.6	44.4	
Primary	48.8	51.2	
Secondary	43.5	56.5	
Superior	57.1	42.9	
Job ^b			0.2
Yes	48.2	51.8	
No	57.4	42.6	
Toxic habits ^b			0.2
Yes	51.3	48.7	
No	41.8	58.2	
Psychiatric history ^b			0.06
Yes	73.7	26.7	
No	48.7	51.3	
Criminal history ^b			0.2
Yes	56.7	43.3	
No	48.4	51.6	
The aggressor state at the time of attack ^b			0.8
Normal	48.8	51.2	
Use of toxic or alcohol	50.8	49.2	

^a Of woman.^b Of the abuser.

The study of factors associated with the occurrence of multiple spousal assault was carried out by univariate logistic regression (Table 5) and multivariate. We have introduced in the multivariate model variables that were associated with significantly statistically or were close to significance ($P < 0.3$). In univariate analysis, only the income of the couple was statistically significant. For multivariate regression, none of the introduced variables were statistically significant.

Discussion

Although domestic violence was considered a universal phenomenon for many authors, it is still a problem that concerns some patriarchal structures that involve women's dependent status against a dominant partner in the family and also in society [1,2]. Women's profile victims of violence in our work answers this concept. Indeed, Moroccan women victims of CV

Table 4 Comparison of daily violence versus violence taking place many times per month depending on woman's and her aggressor's characteristics, and the marriage type.

Characteristic	Daily violence (Women (N) noticed %)	Violence one to many times per month (Women (N) noticed %)	P
Victim's age			0.01
< 25 years	26.5	73.5	
25–35	7.5	92.5	
> 35	18.9	81.1	
Victim's educational level			0.04
Illiterate	31.8	68.2	
Primary	11.8	88.2	
Secondary	11.5	88.5	
Victim's job			0.7
Yes	14.4	85.6	
No	16.2	83.8	
Couple's wage			0.04
≤ GMW	25	75	
> GMW	8.3	91.7	
Children's Number			0.15
≤ 2	13.7	86.3	
> 2	21.8	78.2	
Marriage type			0.9
By love	15.7	84.3	
Family arrangement	16	84	
By necessity	14.3	85.7	
Aggressor age			0.9
< 25 years	20	80	
25–35	19	81	
> 35	14.1	85.9	
Aggressor's educational level			0.4
Illiterate	29.8	79.2	
Primary	16.2	83.8	
Secondary	11.9	88.1	
Superior	33.3	66.7	
Aggressor's job			0.4
Yes	14.2	85.8	
No	18.6	81.4	
The aggressor state at the time of attack			0.6
Normal	18.4	81.6	
Use of toxic or alcohol	15	85	

were predominantly young, with a low or medium educational level and no job in two thirds of the cases. They belonged to families with a mean average socioeconomic level. What is also reported by most international studies [1,8]. As for the aggressor, he was the spouse in almost all cases. This is explained by the fact that marriage is the only relationship allowed between man-woman in our Arab-Muslim society context. This attacker is from low/medium educational and economic level and essentially with toxic habits, which is also reported by most authors [9].

In this study, half of women victims of CV were married after starting a love relationship before marriage with their partners. From these results, we can conclude that even a loving commitment before marriage does not protect women suffering from violence in the private sphere.

In our sample, the predominant type of violence is physical one that was reported by our patients, this is explained by the fact that the study was held in a forensic unit receiving

predominantly female victims of attack and injury and/or sexual abuse. This violence is associated with emotional abuse in 45% of the cases and sexual abuse in 5% of the cases.

According to the WHO report in 2002 [10] and most studies [1], the most common couple violence type is the psychological violence. In Brazil, the prevalence of emotional abuse in a study was 53.8%, followed by physical abuse (32.2%) and sexual abuse comes last with a prevalence rate of 12.4% [8].

In our study, domestic violence began in more than two thirds of the cases early during the first year of marriage; it coincided largely with pregnancy or adultery. Violence was repeated in more than 80% of the cases one or more times per month. Against such violence, almost half of the women did not take any legal action. These results show that women continue to maintain their marriage because of their financial dependence on their spouses, their fear for the children's future in the hope of improvement and change of their spouses and also because of the surrounding environment

Table 5 Logistic regression analysis of the prevalence of multiple domestic violence.

	β	Wald statistic	P value	Odds ratio	IC	
					Lower bound	Upper bound
Victim's age	−0.152	0.739	0.390	1.165	0.823	1.645
Victim's educational level	−0.170	2.306	0.129	0.843	0.677	1.051
Marital status	−0.412	0.715	0.398	0.663	0.255	1.720
Victim's job	−0.129	0.245	0.621	0.879	0.528	1.464
Victim's toxic habits	0.486	0.430	0.512	1.626	0.380	6.955
Children's number	−0.172	0.344	0.558	0.842	0.473	1.497
Couple's wage	−0.928	8.369	0.004	0.395	0.211	0.741
Aggressor's age	0.029	0.014	0.906	1.030	0.634	1.672
Aggressor's educational level	−0.091	0.478	0.489	0.913	0.704	1.183
Aggressor's job	−0.103	0.743	0.389	0.902	0.714	1.140
Aggressor's toxic habits	0.077	0.053	0.819	1.080	0.558	2.091
Aggressor's criminal history	−0.334	1.243	0.265	0.716	0.398	1.288
Aggressor's psychiatric history	−1.062	3.152	0.076	0.346	0.107	1.117
The aggressor state at the time of attack	0.077	0.053	0.819	1.080	0.558	2.091

impact. In our study, the reason most frequently reported by women of the violence is the aggressiveness of the husband followed by economic reasons, as shown in Table 2.

More than three-quarters of women expressed that their husbands had toxic habits and more than two thirds said that the episode of violence occurred while the spouse was taking alcohol and/or drugs. Other studies have also underlined that alcohol and drug consumption can incite violence and influenced its frequency of occurrence [4]. The children witnessed the abusive episode on their mothers in almost all cases (93%) and at the same time beaten by their fathers in 66.5% of the cases. The increased risk of violence among children of beaten women risk was indeed reported by most authors in a Chypre study, 70% of men who beat their wives also beat their children [2].

As for factors associated to the occurrence of CV, a Spanish study reported that low income was associated to all types of aggression with the exception of emotional or psychological abuse; women with more than two children and separated women are more likely to report multiple types of abuse [1]. In our work, there was an association between low income (< minimum wage) and the occurrence of physical violence only against physical abuse associated to other types of violence.

We have also noted an association between women under 25 years, that is to say younger, without any educational level, having an income less than the national minimum wage and the daily occurrence of violence.

Due to the fact that this work was held among consultants in forensic unit has limited the possibility of generalizing our results across all of the Moroccan population. Nevertheless, our study allowed us to have an idea about women's profile victims of CV and their attackers in the city of Casablanca, the economic capital of the Kingdom.

Conclusion

This study focused exclusively on the study of the characteristics of woman victim of domestic violence, the

abuser and the abuse suffered and the association between violence with some factors. It comes out that domestic violence affects younger women, from low socioeconomic and educational level, in their marital home by a spouse who takes toxic in most of the cases. Knowing women's profile, survivors of domestic violence is only the first step to better understand the phenomenon; the risk factors in our Moroccan context remain to be determined by appropriate research, in order to suggest action programs aimed to reduce the prevalence of domestic violence in Morocco.

Disclosure of interest

The authors declare that they have no competing interest.

References

- [1] Ruiz-Perez SA, et al. Sociodemographic associations of physical, emotional, and sexual intimate partner violence in Spanish women. *AEP* 2006;5(16):357–63.
- [2] Petroula M, Mavrikioua, Martha Apostolidoub, Stavros K. Parla-liscaSchool. Risk factors for the prevalence of domestic violence against women in Cyprus. *Soc Sci J* 2014;51:295–301.
- [3] Violence against women a priority health problem. Geneva: WHO; 1997.
- [4] Gunay Balci Y, Ayranci U. Physical violence against women: Evaluation of women assaulted by spouses. *J Clin Forens Med* 2005;12:258–63.
- [5] Vander Ende E, Yount KM, Dynes MM, Sibley LM. Community-level correlates of intimate partner violence against women globally: a systematic review. *Soc Sci Med* 2012;75:1143–55.
- [6] National survey of the prevalence of violence against women. Report of the High Commissioner for Planning, August 2011 Accessed on the internet: http://www.hcp.ma/downloads/Violence-a-l-egard-des-femmes_t13077.html.
- [7] Guide standards and management standards for women and children victims of violence. Morocco; 2007.
- [8] Mathias AKRA, Bedone A, Osis MJD, Fernandes AMS. Perception of intimate partner violence among women seeking care in the

primary healthcare network in São Paulo state, Brazil. *Int J Gynecol Obstet* 2013;121:214–7.

- [9] Klostermann KC, Fals-Stewart W. Intimate partner violence and alcohol use: exploring the role of drinking in partner violence and its implications for intervention. *Aggr Violent Behav* 2006;11:587–97.
- [10] World Report on Violence and Health of the World Health Organization; 2002.